Clinical Study Summary Report (CSSR) Document Status: Draft | **Version:** 1.0 | **Date:** 03-Apr-2026

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1. Administrative & Study Identification

Official Study Title: A Phase Ib, Open-Label, Single-Arm, Multicenter Trial Evaluating the Pharmacokinetics, Safety, and Efficacy of Cevostamab in Chinese Patients With Relapsed or Refractory Multiple Myeloma **Full Study Title:** A Study Evaluating the Pharmacokinetics, Safety, and Efficacy of Cevostamab in Chinese Participants With Relapsed or Refractory Multiple Myeloma **Short Title/Acronym:** Cevostamab Phase Ib RRMM China **Protocol/ Posting ID:** 268959440555904783 **NCT Number:** NCT06934044 **Trial Status:** Recruiting **Sponsor/Company Name:** Hoffmann-La Roche / Roche **Investigational Product:** Cevostamab (Anti-FcRH5/CD3 T-cell engaging bispecific antibody) **Phase of Development:** Phase 1 / Phase Ib **Disease Area:** Multiple Myeloma **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the pharmacokinetics (PK), safety, and tolerability of cevostamab in Chinese participants with relapsed or refractory (R/R) multiple myeloma (MM). **Secondary Objectives:** To evaluate preliminary efficacy (including overall response rate and progression-free survival) and the immunogenicity profile (anti-drug antibodies) of cevostamab in the target population.

2.2 Background & Rationale To address the critical unmet medical need for effective therapies in Chinese patients with relapsed or refractory multiple myeloma who have progressed on standard lines of treatment. Cevostamab is a novel IgG1-based bispecific antibody targeting the FcRH5 receptor on myeloma cells and the CD3 receptor on T cells, facilitating efficient synapse formation and T-cell-mediated destruction of tumor cells. This trial will define its PK and safety profile specifically within the Chinese patient population.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target N: 20 participants **Number of Sites:** Multicenter (Sites across China) **Estimated Study Start:** 13-May-2025 **Estimated Primary Completion:** TBD (Follow-up driven by disease progression)

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Documented diagnosis of Multiple Myeloma (MM) based on standard International Myeloma Working Group (IMWG) criteria. 2. Evidence of progressive disease based on investigator's determination of response by IMWG criteria on or after their last dosing regimen, with current relapsed or refractory (R/R) disease status. 3. Eastern Cooperative Oncology Group (ECOG) Performance Status of 0 or 1. 4. Life expectancy of at least 12 weeks. 5. Agreement to protocol-specified assessments, including bone marrow biopsy and aspirate samples as detailed in the protocol. 6. Resolution of AEs from prior anti-cancer therapy to Grade ≤ 1 or better. 7. Female participants of childbearing potential: Agreement to remain abstinent or use contraception during the treatment period and for at least 5 months after the final dose of cevostamab and for 3 months after the last dose of tocilizumab. 8. Male participants: Agreement to remain abstinent or use a condom, and refrain from donating sperm during treatment and for at least 2 months after the final dose of tocilizumab (if applicable).

4.2 Exclusion Criteria 1. Unable to comply with protocol-mandated hospitalization. 2. Pregnancy or breastfeeding, or intention of becoming pregnant during the study or within specified post-treatment windows. 3. Prior therapy constraints: Prior treatment with cevostamab or same target agent; prior monoclonal antibody, radioimmunoconjugate, or ADC within 4 weeks; prior systemic immunotherapies (e.g., anti-CTLA-4, anti-PD-1/PD-L1); prior CAR-T within 12 weeks; known immune-mediated AEs from prior checkpoint inhibitors; radiotherapy, chemotherapy, or other anti-cancer agent within 4 weeks or 5 half-lives; autologous SCT within 100 days; prior allogeneic SCT or solid organ transplant. 4. Medical history constraints: History of autoimmune disease, progressive multifocal leukoencephalopathy (PML), severe allergic reactions to mAb therapy, amyloidosis, current or past CNS disease (stroke, epilepsy, CNS vasculitis, MM CNS involvement), HLH or MAS, Grade ≥ 3 CRS or ICANS with prior bispecific therapies, or major surgery within 4 weeks. 5. Organ/System constraints: Significant cardiovascular disease limiting a potential response to CRS; symptomatic active pulmonary disease or requiring supplemental oxygen; lesions near vital organs risking sudden deterioration during a tumor flare. 6. Infections: Active bacterial, viral, fungal, etc., requiring IV antimicrobials within 14 days; active symptomatic COVID-19 or requiring IV antivirals within 14 days; positive EBV PCR or CMV PCR; chronic active EBV; positive HBV, HCV, or HIV. 7. History of other malignancy within 2 years (with standard low-risk exceptions). 8. Administration of a live, attenuated vaccine within 4 weeks. 9. Systemic immunosuppressive medications (except corticosteroids ≤ 10 mg/day prednisone equivalent) within 2 weeks. 10. History of illicit drug or alcohol abuse within 12 months prior to screening. 11. Any medical condition or laboratory abnormality precluding safe participation.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Cevostamab administered via step-up dosing to a specified target dose to mitigate cytokine release syndrome (CRS) risks. **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Administered in 21-day cycles until documented disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care, including standard premedication (e.g., corticosteroids, antipyretics) and intervention agents like tocilizumab (Actemra) for the mitigation and treatment of CRS. **Prohibited:** Concurrent investigational agents, targeted MM therapies, or prior FcRH5-directed therapies.

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6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Disease Assessment, Baseline Labs, Bone Marrow Biopsy/Aspirate
Baseline	Cycle 1 Day 1	First Dose (Step-up), Intensive PK sampling, Vital Signs, Safety Observation, Hospitalization as mandated
Interim	Subsequent Cycles	Safety Labs, Efficacy Assessment (IMWG criteria), ADA monitoring
End of Study	30+ days post-last dose	Final Safety Assessments, AE Resolution Follow-up, Survival/Contraception Follow-up

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. Serum Concentration of Cevostamab at Specified Timepoints. 2. Percentage of Participants with Adverse Events.

7.2 Secondary & Exploratory Endpoints 1. Objective Response Rate (ORR). 2. Rate of Complete Response (CR) or Better. 3. Rate of Very Good Partial Response (VGPR) or Better. *(Additional exploratory endpoints may include Duration of Response (DoR), Progression-Free Survival (PFS), and Overall Survival (OS)).*

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Intensive monitoring for mechanism-based toxicities including Cytokine Release Syndrome (CRS) and Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS). **Serious Adverse Events (SAEs):** Standard reporting timeline (typically within 24 hours of investigator awareness).

Data Monitoring Committee (DMC): Internal Safety Review Committee (SRC) established to evaluate step-up dose safety, dose-limiting toxicities, and cohort clearance.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving ≥ 1 dose of cevostamab) and PK-evaluable Set. **Sample Size Justification:** A target enrollment of 20 participants is driven by Phase Ib descriptive and precision-based standards for identifying PK parameters and detecting high-frequency safety signals. **Handling of Missing Data:** Missing data will not be imputed for safety endpoints. Standard censoring methodology will be applied for time-to-event efficacy analyses.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Scheduled onsite and remote clinical monitoring visits to assure data integrity and patient safety. **Audit Trail:** Electronic Case Report Form (eCRF) data entry with digital audit trails and managed data clarification forms.

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11. Study Identifiers & Governance

Primary ID: 268959440555904783 **Registry Number:** NCT06934044 **Sponsor/Lead Organization:** Roche / Hoffmann-La Roche **Study Title:** A Study Evaluating the Pharmacokinetics, Safety, and Efficacy of Cevostamab in Chinese Participants With Relapsed or Refractory Multiple Myeloma **Official Regulatory Title:** A Phase Ib, Open-Label, Single-Arm, Multicenter Trial Evaluating the Pharmacokinetics, Safety, and Efficacy of Cevostamab in Chinese Patients With Relapsed or Refractory Multiple Myeloma

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12. Clinical Framework (Multiple Myeloma Specific)

Primary Condition: Relapsed or Refractory Multiple Myeloma (RRMM) / Multiple Myeloma, Drug Therapy **Diagnosis Threshold:** Refractory to or relapsed following prior standard lines of therapy (progressive disease per IMWG). **Medical Methodology:** T-cell engaging bispecific antibody therapy (CD3 x FcRH5) **Optimization Target:** Target dose achievement with a manageable safety profile (specifically mitigating CRS/ICANS)

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13. Study Procedures & Methodology

Management Pathway: Step-up dosing administration protocol to safely manage initial T-cell activation. Protocol-mandated hospitalization may be required. **Education Protocol (HCP):** Specialized training on the grading and rapid management of CRS and neurotoxicity (including use of tocilizumab). **Education Protocol (Patient):** Proactive education on reporting early signs of infection and CRS symptoms, alongside rigorous contraception requirement guidance. **Quality Audit Mechanism:** Safety run-in evaluation, regular pharmacovigilance tracking, and continuous clinical review.

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14. Observation & Follow-Up Schedule

Total Duration: Follow-up continues until disease progression, death, or study withdrawal. **Onsite Visit Cadence:** Intensive monitoring and hospitalization requirements during Cycle 1, followed by visits matching the treatment cycle. **Remote Monitoring Frequency:** Periodic safety checks between cycles as required by the clinical site. **Checklist Review Interval:** Every 21-day cycle.

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	**[DD-MMM-YYYY]**
Clinical Operations Lead	**[Name]**	___	***[DD-MMM-YYYY]***
Lead Statistician	***[Name]***	_____	[DD-MMM-YYYY]